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Special Section

BEHAVIORAL THERAPY DEVELOPMENT AND PSYCHOLOGICAL SCIENCE: Reinforcing the Bond

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Abstract—*An abundance of research from diverse areas of psychological science is potentially relevant to behavioral therapy development research. The National Institute on Drug Abuse of the National Institutes of Health convened a workshop to reinforce the connection between basic behavioral and therapy development research. The articles in this Special Section are the product of this workshop. Several lines of basic behavioral research are described, and implications for the development of behavioral therapies are discussed. This Special Section highlights the notion that strengthening the bond between behavioral therapy development research and basic behavioral science will accelerate the advancement of knowledge about behavior, behavioral change, and therapeutic interventions.*

The scientist-practitioner model of clinical psychology assumes an intimate link between the practice of psychology and science. Lamenting the failure of the scientist-practitioner model to become more of a reality than an ideal, Beutler, Williams, Wakefield, and Entwistle (1995) have commented that researchers often fail to seek and find worth in clinicians' perspectives. Stricker and Trierweiler (1995) have stated that "the name, *scientist-practitioner model* seems to have become merely a rhetorical device reserved for the scientific training programs" (p. 995).

However, the scientist-practitioner model must be more than just rhetoric among behavioral therapy researchers if knowledge about behavioral therapies is to advance. (The term behavioral therapy is used here in the broadest sense, and refers to psychosocial therapeutic interventions, including operant and classical behavior therapy, cognitive therapy, and other forms of psychotherapy.) Not only have many behavioral therapy researchers been trained in the Boulder model (Raimy, 1950), they embody it. They have identities as both scientists and clinicians. As clinicians working with clients, they derive many of their ideas about therapy from doing therapy. As researchers, they place a high value on conducting rigorous studies of behavioral therapies and the theories on which they are based, and on using therapeutic interventions that are supported empirically.

Within multiple areas of psychology, there is currently an abundance of basic research being conducted that could positively influence behavioral therapy development research. The fields of cognitive, social, personality, developmental, and experimental psychology, for example, all have produced research that could directly affect behavioral therapy development. Many behavioral therapy researchers are designing their studies within the context of research from diverse areas of psychology, and others are actively interested in utilizing any new knowledge that could improve the efficacy of their therapies.

However, sometimes there are barriers imposed between researchers in "nonapplied" areas of psychology and those in behavioral therapy development. For example, the two groups are sometimes physically separated. It is not unusual for behavioral therapy researchers to be situated within medical school or other settings where there is access to clinical populations but less contact with academic psychology departments. Scientists not involved in behavioral therapy research are more likely to be located in nonmedical settings within universities and may have difficulty gaining access to clinical populations. However, the theoretical questions that cognitive, social, experimental, and other psychologists are asking, quite often, can be answered best in highly controlled settings in which the use of clinical populations would be unnecessary or even ill-advised. This is not, in and of itself, a problem except insofar as it contributes to the separation between behavioral therapy researchers and psychological scientists not directly involved in the development of therapies.

Immediate demands for solutions to public-health problems may, at times, function as yet another dividing force. For example, behavioral therapy researchers may feel pressured to find a therapy that works—but no concurrent pressure to find out how or why it works. It is unfortunate when answers to such theoretically grounded questions, our greatest hope for the future, are given short shrift in favor of more immediate goals.

Another difficulty is the sheer volume of potentially relevant psychological science to be considered when developing a behavioral therapy. Although behavioral therapy researchers can and do incorporate knowledge and theory from all aspects of psychological science into their work, it is a daunting if not impossible task to keep up with all potentially relevant research. This is one reason why it is essential that psychologists make special efforts to identify and communicate with one another about research of potential relevance to the development of behavioral therapies.

To encourage and strengthen the opportunities for the exchange of ideas between researchers from all types of psychological science and to direct the attention of this community of researchers toward the issues of behavioral therapy development, a series of workshops is planned by the National Institute on Drug Abuse (NIDA) of the National Institutes of Health. The articles in this Special Section are the product of the first in that series of workshops, "Behavioral Therapy Development and Psychological Science." This first meeting, which we chaired, took place on May 14 and 15, 1996, and was designed to reinforce the bond between behavioral therapy development research and other, less applied, areas of psychology. It was a natural outgrowth of NIDA's Behavioral Therapies Development Program, a program designed to foster and support all stages of research in behavioral therapy development.

Six behavioral scientists presented papers at the workshop, and six behavioral scientists with expertise in behavioral therapy development responded to these papers, highlighting how the research presented may relate to behavioral therapy development. The six presentations were given by Daniel Wegner, Steven Hayes, William Swann, Baruch

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Fischhoff, Timothy Baker, and James Pennebaker. The six discussion leaders who responded to these papers were, respectively, Varda Shoham, Richard McFall, Marsha Linehan, Karla Moras, Kathleen Carroll, and Richard Bootzin. Thomas Borkovec and Gerald Davison provided general commentary and wisdom at the meeting.

This workshop series is particularly timely because a major pragmatic obstacle to linking behavioral therapy development with basic behavioral science has been overcome recently. In the past, funding in behavioral therapy research was essentially limited to the testing of already existing behavioral therapies. However, in 1992, NIDA began funding what it calls Stage I behavioral therapy development research. Stage I research allows an investigator to take ideas from diverse areas of psychological science and translate them into innovative behavioral therapies. Thus, it is now possible for an investigator to obtain funding to develop a novel behavioral therapy, write a manual describing it, and begin to test its efficacy.[1] This funding of Stage I research presents a new opportunity for psychological scientists not previously involved with behavioral therapy development and behavioral therapy development researchers to collaborate or otherwise draw upon ideas from each other to undertake research in behavioral therapy development.

The driving force behind the workshop was the idea that for behavioral therapy development to continue to thrive, strengthening its connection to psychological science is of paramount importance. The articles that follow exemplify the importance of this endeavor, and we believe that many of the ideas presented here will be incorporated into the development of new behavioral therapies. It is our hope that this Special Section helps to further cement the bond between behavioral therapy and psychological science, and that it will become second nature for applied and nonapplied behavioral scientists to look to each other to enrich their scientific and public-health pursuits.

1. NIDA's program announcement, "Behavioral Therapies Development Program" (PA 94-078), supports all stages of behavioral therapy development research, including Stage I, or early therapy development, refinement, and pilot testing; Stage II, or therapeutic efficacy testing and research on the mechanism of action of therapies; and Stage III, or research testing the transferability of efficacious therapies to community settings. A recent addendum to this announcement, "Innovative Stage I Behavioral Therapy Development Research," specifically encourages research, based on basic behavioral science, on the development of creative new therapies and interventions for disorders involving drug use and reducing risk of HIV infection.

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